جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : مبروكه صبرى السيد صالح
المحافظة : المنوفيه **الحي / المدينة :** كفر داود **العنوان :** ---
رقم البطاقة : 28404011803562 **رقم التليفون :** 01062242652 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 01-04-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مبروكه صبرى السيد صالح
National ID: 28404011803562

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

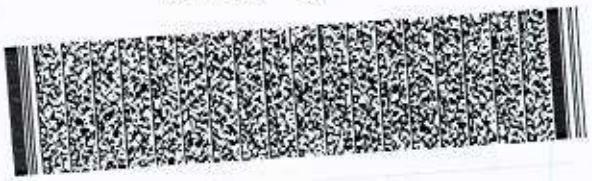
٢٨٤٠٤٠١١٨٠٣٥٦٢ ٢٠٢٤/٠٨

لا تعمل

أنثى مسلمة

١٢

البطاقة سارية حتى ٢٠٣١/٠٨/٠٦



جمهورية مصر العربية

بطاقة تحقيق الشخصية



مبروكه

صبرى السيد صالح

كفر داود

مركز السادات - المنوفية

٢٨٤ ٠٤ ٠١ ١٨ ٠٣٥ ٦٢

١٩٨٤/٠

KR5963204

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 16/4/2025 G

Name	[illegible]	MRN	٢٠٢٢
		ID	
Age	41yr	Phone	01093395770
Menopausal status	Post MenoPause	Marital status	Married
NRS SCORE		Family history	-
BMI		REASON OF MDT	Postoperative
CO morbidity	Pt Received Neadjuvant		

Clinical Data

BX: IDC

ER:	PR:	HER 2:	Ki67:
8/8	8/8	3	

Sonomammography:

Mets Workup: (4) Post MRM, Post Neadjuvant chemotherapy.

R Post-op L

TNM

Recommendations: Pathology Report: Tx, No, Partial pathological Response.

MDT OUTCOME & PLAN Pt was designated Tx on account of hypothesis mass ?!! → review pathology report if reported Non-PCR → ① TCH (14 cycles) ② PORT (if c/w RCM) ③ ET (AI +/- Zoladex) 5 yrs.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: مبروكة صبري السيد **MR. :** 6944	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 65-year-old female with a history of left breast cancer, stage cT4bN3M0, IDC, ER 8/8, PR 8/8, HER2+. The patient underwent left mastectomy with lymph node dissection (LVSI, N0/10). She is currently receiving adjuvant therapy with Kadcyla (trastuzumab emtansine) 260 mg and Femara (letrozole) as part of a hormone-targeted regimen, with Zoladex (goserelin) added for endocrine support. The patient has completed nine cycles of Kadcyla and is scheduled to continue with cycle 10.

The patient's treatment plan includes ongoing Kadcyla 260 mg infusions and oral Femara, with supportive laboratory monitoring. The next cycle of Kadcyla is scheduled following acceptance of the pharmacy visit for C 11 Kadcyla 260 and Femara.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Patient Name	: الأستاذه / مروكه صبرى السيد صالح	Request No.	: 13008644
Patient No.	: 28404011803562	Request Date	: 08/09/2024
Lab No.	: SR-1299/24	Verification Date	: 18/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:
One paraffin block labeled by (201804/24) and three slides were received.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- DCIS: Positive (comedo).
- Angioinvasion: Negative.
- Perineural invasion: Negative.

IHC:
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:
Left breast cancer, Slide revision:
invasive duct carcinoma, grade II, with DCIS (comedo).

IPV:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاستاذه / مروكه صبرى السيد صالح	Request No.	: 13008644
Patient No.	: 28404011803562	Request Date	: 08/09/2024
Lab No.	: SR-1299/24	Verification Date	: 18/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - تغطية الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مبروكه صبرى السيد صالح	Request No.	: 13009873
Patient No.	: 28404011803562	Request Date	: 15/03/2025
Path No.	: 1034/25	Verification Date	: 07/04/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 28x20 cm with covering skin ellipse measured 22x15 cm.
* Tumor extent: A greyish white firm tumor bed site.
* Size: 16x7 cm.
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 2 cm away from deep margin.
* Attached axilla :Fibrofatty tissue measured 8x7cm entangling 10 L.Ns up to 1.5x1 cm .

Microscopic Examination:
* Histological type: No residual tumor .
* Lymphovascular invasion: Minimally positive.
* Ductal carcinoma in situ: No.
* Skin involvement: Free nipple .
* Lymph node involvement: Free lymph nodes (0/10)
* Resection Margins: Free deep margin.
* Histologic response to therapy: Pathological partial response.

Diagnosis:
Left breast cancer post NACT, MRM specimen:
No residual tumor remnants.
- Positive minimal lymphovascular invasion.
- Free deep margin.
- Free nipple.
- Free lymph nodes.

Verified by :
Dr.Menan Hossam

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / مبروكه صبرى السيد صالح	Request No.	: 13009873
Patient No.	: 28404011803562	Request Date	: 15/03/2025
Path No.	: 1034/25	Verification Date	: 07/04/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- Pathological partial response.
- Stage: ypTx, ypN0.

Comment:
There is no invasive tumor or DCIS in breast tissue , yet there was very few positive lymphovascular invasion so staged as (Tx). Lymphvascular imvasion is considered bad prognostic factor.

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :
Dr.Menan Hossam

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة .. القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH
GENDER: Female
DATE : 19-Aug-2024 11:16 AM

PATIENT ID: 20224011103
AGE : 40 y
ORDER: Z109332,Z109333

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTAL ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view
- Complementary ultrasound

REPORT:

- ACR:D

Left breast:

- UIQ posterior third indistinct dense lesion with overlying suspicious microcalcification extending anteriorly in linear pattern measuring 6.6X2.2 cm, about 9 cm from the nipple corresponding by US to 11 o'clock irregular hypoechoic lesion measuring 3.3X1.6 cm, about 0.6 cm depth from the skin.
- Multiple (more than 3) axillary level I lymph nodes, measuring 2.7X2 cm.
- no infra or supraclavicular lymph nodes.

Right breast:

- UIQ posterior third focal assymetry corresponding to normal glandular tissue.
- No spiculated mass lesions or grouped microcalcifications.
- No solid or cystic mass lesions.
- Normal skin thickness and contour
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left breast UIQ malignant mass lesion with ipsilateral axillary level I lymphadenopathy (BIRADS 5) for histopathological correlation.
- Right breast UIQ focal assyemtry corresponding to normal glandular tissue (BIRADS 3).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yomna Tarek, MSc 19-Aug-2024 10:43 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH
GENDER: Female
DATE : 01-Sep-2024 09:[illegible] AM

PATIENT ID: 20224011103
AGE: 40 y
ORDER: 3012125,3012126

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC ASSESSMNET OF THE LIVER

1- CHEST:

- Left breast UIQ enhancing mass lesion measuring 22 x 20 mm associated with diffuse parenchymal edema and mild skin thickening.
- Multiple pathologically enlarged left axillary lymph nodes level I & II lymph nodes; the largest is at level I measuring 22 x 23 mm.
- Subcentimetric right axillary level I and II lymph nodes are also noted.
- Few scattered tiny pulmonary nodules; the largest is the right upper lobe posterior segment measuring 3 mm in diameter.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- Mildly enlarged liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys (apart from few bilateral tiny calyceal stones; the largest is left middle calyceal measures 3 mm in diameter), adrenals, aorta and I.V.C.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Left adnexal cyst measures 18 x 28 mm
- Minimal pelvic free fluid is noted likely functional.
- No pathologically enlarged abdominal or pelvic lymph nodes.

The revised bone window revealed:

CT Scan
[[DOCTYPE:CT_SCAN]]

1. Dorso-lumbar spondylo-degenerative changes with related sclerosis of VEPs.
2. Right femoral head and left iliac bony tiny sclerotic foci likely representing bone islands.
3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

OPINION:

- **Left breast neoplastic process with ipsilateral pathological axillary lymphadenopathy.**
- **Few tiny pulmonary nodules for follow up.**
- **No evidence of hepatic metastatic deposits.**

Reported By,
Toka Elbehairy,MSc
Dr. Salwa Yahya, MD 03-Sep-2024 11:17 PM

www.Baheya.org 16602
CT Scan
[[DOCTYPE:CT_SCAN]]

B.HEYA

بهية

PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH
GENDER: Female
DATE : 19-Aug-2024 11:16 AM

PATIENT ID: 20224011103
AGE: 40 y
ORDER: Z109332,Z109333

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
- Screening.

TECHNIQUE:
- CC and MLO View.
- Complementary ultrasound

REPORT:
- ACR:D

Left breast:
- UIQ posterior third indistinct dense lesion with overlying suspicious microcalcification extending anteriorly in linear pattern measuring 0.6X2.2 cm, about 9 cm from the nipple corresponding by US to 11 o'clock irregular hypoechoic lesion measuring 3.3X1.6 cm, about 0.6 cm depth from the skin.
- Multiple (more than 3) axillary level I lymph nodes, measuring 2.7X2 cm.
- no infra or supraclavicular lymph nodes.

Right breast:
- UIQ posterior third focal assymetry corresponding to normal glandular tissue.
- No calculated mass lesions or grouped microcalcifications
- No solid or cystic mass lesions.
- Normal skin thickness and contour
- Normal nipple areola complexes

OPINION:
- Left breast UIQ malignant mass lesion with ipsilateral axillary level I lymphadenopathy (BIRADS 5) for histopathological correlation.
- Right breast UIQ focal assymetry corresponding to normal glandular tissue (BIRADS 3).

RECOMMENDATION:
Referral to Onc

Reported By,

Dr. Yomna Tarek, MSc 19-Aug-2024 10:43 AM

www.BabyA.org 16602

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH GENDER: Female DATE : 09-Sep-2024 12:20 PM	PATIENT ID: 20224011103 AGE: 40 y ORDER: 3026825

99MTC-MDP BONE SCAN

Clinical Presentation: -
40-year-old female with known left breast cancer, for skeletal survey as part of IS metastatic work-up.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner.

Findings:

- No hot/cold focal areas of abnormal tracer accumulation that would account for osseous deposits could be detected allover the surveyed skeleton.
- Mild degenerative changes of the dorsal vertebrae.
- Mild arthritic changes of the knees.
- Warm soft tissue uptake in the region of the left breast: known site of primary malignancy.
- Both kidneys are normally visualized.

Conclusion:

- No scintigraphic evidence of metastatic bone lesions all-over the surveyed skeleton.

Reported By,

Dr. ElShaymaa Hussein, Nuclear Medicine Consultant 10-Sep-2024 01:46 PM

Bone Scan
[[DOCTYPE:BONE_SCAN]]

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

MR 3023/24

Name: مبروكة صبري
Date 21/9/2024

EF	63%	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	3.4	2-3.7cm
LVESD	3.4	2-4cm	LA	3.7	2-4cm
FS	32%	25-45%	RV		0.7-2.3cm(supine)
SWT	1.09	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA No SWMA at rest

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion No

D/Mohamed Ali

Msc cardiology

Echo
[[DOCTYPE:ECHO]]





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]